



Procedure

Community Program for Opioid Pharmacotherapy (CPOP) – Continuation in Hospital

Reference #:MMP003

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Medical, Nursing, Pharmacy

Introduction

Opioid Substitution treatment (OST) is made available to opioid dependent individuals in Western Australia through the Community Program for Opioid Pharmacotherapy (CPOP).

Prescribing of OST for CPOP is restricted to continuation of confirmed, current CPOP therapy for the duration of admission, provided it is safe to do so.

- Prescribers working in a SCGOPHCG role are NOT authorised to initiate (including re-initiating) OST regardless of their authorised CPOP prescriber status.

The primary aim of these recommendations is to avoid overdose with OST.

Table 1: CPOP/OST treatments

Brand / Ingredient(s)	Formulation	Strengths Available
Methadone pods	oral liquid	50 mg in 10 mL of oral liquid
Subutex® (buprenorphine)	sublingual tablet	400 micrograms 2 mg 8 mg
Suboxone® (buprenorphine / naloxone)	sublingual film	2 mg / 0.5 mg 8 mg / 2 mg
Buvidal ® (buprenorphine)	depot injection	Weekly: 8 mg, 16 mg, 24 mg, 32 mg Monthly: 64 mg, 96 mg, 128 mg, 160 mg
Sublocade® (buprenorphine)	depot injection	100 mg and 300 mg monthly

Risk Statement

Non-compliance with this procedure will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☒

Breach SCGOPHCG Mission & Values	☒	Staff Safety	☒
Other:	☐		

Definitions

Authorised CPOP prescriber	A Medical Practitioner that has completed specific CPOP training and is authorised by the Department of Health (DOH) to treat each individual patient
CPOP Advice and Support	CPOP Advice and Support provides advice on clinical management of CPOP patients. It is a 24 hours-a-day 7 days-a-week telephone service available to health practitioners who require support with methadone and buprenorphine treatment across metropolitan and regional WA. Telephone 9442 5042 or free call 1800 688 847 outside the Perth metropolitan area
Community Program for Opioid Pharmacotherapy (CPOP)	CPOP treatment is an effective strategy for helping opioid dependent individuals reduce their opioid use and stabilise their lives by reducing the demand for opioids, and the resulting harms. Treatment is provided via CPOP, a partnership between the government and community service providers. The program adopts a shared care approach, with CPOP treatment provided through general practices, Next Step and other drug and alcohol treatment services, correctional institutions and community pharmacies
Opioid Substitution Treatment (OST)	The purpose of OST is to replace use of the problem drug (such as heroin, morphine or other opioid medication) with a long-acting medication within the boundaries of a comprehensive treatment strategy. Substitution treatment frees opioid dependent individuals from the time consuming and costly burden of procuring opioids through subterfuge or unlawful means on a daily basis. They may then function in a more 'normal', pro-social manner, thus having improved capacity for employment, study, repairing relationships, addressing legal matters, finding suitable accommodation and addressing medical and mental health issues. During maintenance treatment the individual may address the underlying issues which resulted in substance dependence in the first place. Both methadone and buprenorphine maintenance therapies have been demonstrated to reduce crime, transmission of blood borne viruses, intravenous drug use, and opioid overdose rates.
S8 register	The WA Health Register of Drugs of Addiction (HA 14). Where all controlled drug transactions in clinical areas are recorded as per the Medicines and Poisons Act 2014
'in treatment'	For the purposes of CPOP, 'in treatment' means: • A prescriber is authorised to treat the person • The person has a current OST prescription • The person has taken their doses in accordance with the prescription
Takeaway doses	Community treatment of opioid dependence with methadone or buprenorphine is based on daily, supervised dosing at a pharmacy. However, in the community, some well stabilised patients with particular needs may be prescribed some of their doses as 'takeaway' doses. Takeaway doses are dispensed to the patient by their dosing pharmacy, using a standardised procedure.

	Patients discharged from hospital are never to be dispensed takeaway doses under any circumstances.
WA HMC	WA Hospital Medication Chart - previously known as the National Inpatient Medication Chart (NIMC)

Procedure

When a patient participating in CPOP is admitted to hospital and continuation of CPOP treatment is required, it is important that doses are prescribed and supplied in accordance with all relevant policies, procedures and legislation.

This procedure outlines the requirements for management of admitted CPOP patients to:

- Minimise risk of adverse outcome due to overdose
- Ensure continuation of prescribing is legal
- Limit risk associated with diversion of CPOP treatments

Opioid withdrawal is less harmful to the patient compared to overdose, particularly if alcohol or illicit drugs (including other opioids) have been used prior to presentation

1.0 Dose confirmation and documentation

Independent verification of dosing information must occur prior to administering the first dose of OST in hospital to avoid 'double dosing' and the risk of overdose.

Useful Contact Numbers and Internet Pages	
Community Program for Opioid Pharmacotherapy (CPOP)	☎ (08) 9222 6812 (Department of Health, Medicines and Poisons Regulation Branch, for general enquiries)
Community Pharmacotherapy Program (CPP)	☎ (08) 9219 1907 Monday to Friday 08:30am to 4:30pm (except public holidays).
CPOP Advice and Support	☎ (08) 9442 5042 or 180 0688 847 operates a 24/7 phone service for health professionals providing clinical advice
Next Step Clinic Drug and Alcohol Services	☎ (08) 92191919 https://www.mhc.wa.gov.au
SCGOPHCG Alcohol and Drug Service	DECT 76323 Available: Monday to Friday 07:00am to 6:00pm Saturday and Sunday: 08:00am to 4:00pm (except public holidays) SCGH.ADS@health.wa.gov.au

1.1 Confirm community prescription is valid, current dose and doses supplied

Liaison and dose confirmation can be undertaken by:

- The SCGOPHCG treating doctor, or
- The Clinical Pharmacist, or

- The Alcohol and Drug Service - CNC and Clinical Nurses.

Only provide treatment after **all** relevant details below have been confirmed. Even where a patient has a current OST prescription, it must be determined whether they have been taking their doses and supplied any takeaway doses before they can be determined to be 'in treatment'.

1. Obtain the patient's CPOP community pharmacy name and telephone number.
 - It is advisable to use the telephone directory to verify the provided contact details.
 - Request a copy of the current prescription and dosing record from the dispensing CPOP community pharmacy.
 - The details of buprenorphine depot injections will require verification with the CPOP prescriber at the clinic where it is administered.
2. For patients dosing at the outpatient Next Step Clinic, this service can provide confirmation.
3. Check if takeaway doses have been dispensed, or if more than one pharmacy is involved:
 - Some patients are authorised to have CPOP treatment dispensed at more than one pharmacy (e.g. if their usual pharmacy is closed on Sunday and public holidays) or are allowed a 'takeaway' dose(s) under certain circumstances.
 - If 'takeaways' have been supplied they must be accounted for before further doses can be prescribed or administered.
 - Note: patients presenting with 'takeaway' methadone doses will have the liquid volume diluted by community pharmacy (as per CPOP protocol) and therefore the strength appearing on the label may be different from commercial product.

1.2 If the patient's prescription has recently expired or is likely to expire during the admission, contact the CPOP prescriber in the community to notify and to verify the intention to continue therapy. Document in the medical notes

The following details **must be confirmed and documented** in the inpatient medical notes:

1. Patient name, date of birth, hospital record number (i.e. 3 approved identifiers).
2. CPOP prescriber identity.
3. Current dose in both mg and mL for methadone and mg for Suboxone®, Subutex® and buprenorphine depot injections.
4. Script expiry date
5. Frequency of dosing, date and time of the last administered dose. For depot injections, the specific site of administration should be verified.
6. If the patient has 'takeaway' dose arrangements, document the number of doses, if any dispensed and when last supplied, accounting for any remaining/unaccounted doses.
7. Name of the pharmacist spoken to, pharmacy name and contact details.
8. If more than one pharmacy is involved, follow steps 1 to 6 to record all details for each separate pharmacy.
9. A copy of the current prescription and dosing record should, where possible, be requested as a verification of the dose.

1.3 After-hours admissions

- Dose confirmation and dosing can wait until the following morning as per the WA Medicines Handling Policy.

- Missed doses are not considered a medical emergency. The long half-lives of methadone and buprenorphine mean missing a single dose is unlikely to cause significant physical discomfort.
- If supply is deemed urgent, CPOP Advice and Support can be contacted after obtaining the mandatory information detailed above (see [1.1](#) and [1.2](#)) to provide advice on management.

1.4 Special considerations

- A patient currently prescribed OST is considered to be at significant risk of overdose if:
 - They are in the induction phase of treatment and have missed a single OST dose, or
 - They are in the maintenance phase of treatment and have not received an oral OST dose for 4 or more days, or
 - They have not received their depot buprenorphine injection within the recommended treatment interval.
- If the patient has missed doses, OST is not to be prescribed until the hospital treating doctor has consulted with the patient's CPOP prescriber or has contacted CPOP Advice and Support if the usual CPOP prescriber is unavailable (see below red box 'Missed doses').
- Where OST is being administered by depot injection, once the patient is stable, there is some flexibility in when the next dose needs to be administered e.g. Buvidal® monthly depot can be administered up to a week after the dose was due. This means dosing may be able to be delayed until the patient is discharged from hospital. Conversely, some patients being treated with depot injections may also require supplemental doses for cravings if clinically indicated. Advice from the patient's CPOP prescriber or CPOP Advice and Support should be sought in both circumstances.

2.0 Prescribing

2.1 Prescribe on WA HMC

All OST orders must be prescribed on the WA HMC (see Table 2 for examples). There are different strengths of sublingual tablets and films available, and a patient may be on a combination of multiple strengths to make up their prescribed dose (see Table 2).

- CPOP treatment is usually administered in the morning as a single dose. Subutex® or Suboxone® dosing may be less frequent (every second or third day).
- The Clinical Pharmacist can provide guidance regarding this process.

MISSED DOSES

1. If a patient has not received a dose of methadone or buprenorphine CPOP treatment (Subutex® or Suboxone®) within the past 72 hours, **do not prescribe CPOP treatment until the patient's CPOP prescriber has been consulted**, as dose adjustment may be necessary.
2. If a patient is in the induction phase (first 1 to 2 weeks of treatment) or re-initiation phase, and has missed a single dose, then do not prescribe CPOP treatment until the patient's CPOP prescriber has been consulted.
3. If a patient has not received their weekly Buvidal® depot injection for more than 14 days, or their monthly depot buprenorphine injection for more than 8 weeks **do not prescribe CPOP treatment until the patient's CPOP prescriber has been consulted**, as dose adjustment may be necessary.

2.2 Special prescribing considerations

- It is usual for CPOP treatment to continue during hospital admission unless opioids are temporarily contraindicated, or the patient is unable to take medication orally or through nasogastric tube.
 - If alterations to the CPOP treatment regimen are required, the medical officer should consult the Alcohol and Drug Service, patient's CPOP prescriber or the CPOP Advice and Support.

2.2.1 Depot buprenorphine injections

- Once the patient is stable on depot injection OST, there is some flexibility in when the next dose needs to be administered. This may mean that the dose could be delayed until the patient is discharged from hospital. The patient's CPOP prescriber or the CPOP Advice and Support service should be contacted for advice.

2.2.2 Analgesia

- Adequate post-operative or other analgesia should be provided as indicated in addition to the daily CPOP treatment dose, using agents other than methadone, Subutex® and Suboxone® (do not count the CPOP treatment as analgesia).
 - An **Acute Pain Service (APS)** review pre or post-operatively is warranted.
- As buprenorphine in Subutex® and Suboxone® has stronger binding affinity to the Mu pain receptor than other opioids, treatment with other opioids (for pain or opioid reversal) is more complex. Refer to APS or the CPOP Advice and Support team for an inpatient pain management plan.

2.2.3 Methadone and split doses

- Patients on methadone treatment via CPOP must only be given methadone oral liquid (no other formulations are allowed due to the diversion risk). The only exemption is for patients who have been seen by an Acute Pain Specialist and have:
 - An APS pain management plan in place, and
 - Have been prescribed methadone tablets or injections, to facilitate TEMPORARY (<24 hours) splitting of the dose until the CPOP methadone liquid can be supplied by pharmacy.
 - Methadone tablets prescribed under this temporary exemption must be crushed using a commercial tablet crusher and dispersed in 10-20mL of water prior to administration.
- CPOP doses **must not be split into multiple doses (versus once daily dosing) unless** under the direction of an Acute Pain Specialist. The total daily dose must not be increased or decreased without first contacting the patient's CPOP prescriber.

Table 3: Examples of OST prescriptions on the WA HMC

Example A – methadone oral liquid				Example B – methadone oral liquid SPLIT DOSE			
Year 20...23....		DATE AND MONTH →		Year 20...23....		DATE AND MONTH →	
Doctors MUST ENTER administration times				Doctors MUST ENTER administration times			
Start Date	Medicine (print generic name)/form	Tick if slow release		Start Date	Medicine (print generic name)/form	Tick if slow release	
8/5	Methadone 5 mg/mL Liquid		1400	8/5	Methadone 5 mg/mL Liquid		0800
Route	Dose and Frequency	and now enter times →		Route	Dose and Frequency	and now enter times →	
PO	22.5 mg = 4.5 mL	mane		PO	10 mg = 2 mL	bd	
Indication	Pharmacy	Imprest S8 S4R		Indication	Pharmacy	Imprest S8 S4R	
CPOP				CPOP	split dose per APS	2000	
Prescriber signature	Print name	SAC/AAN		Prescriber signature	Print name	SAC/AAN	
V-Good V-Good Not supplied on D/G				V-Good V-Good Not supplied on D/G			

For methadone oral liquid, the order should be written with the dose expressed in both mg and mL e.g. methadone 5 mg/mL oral liquid: 22.5 mg = 4.5 mL each day. This minimises the potential for prescribing and administration errors (i.e. five-fold overdose).

Example C – Suboxone® sublingual film			
Year 20... 23 ...		DATE AND MONTH →	
Doctors MUST ENTER administration times →			
Start Date ... 8 .. 5	Medicine (print generic name)/form Suboxone Sublingual Films	☐ Tick if slow release	1400
Route Sublingual	Dose and Frequency and now enter times → 12 mg/ 3 mg mane		
Indication CPOP	Pharmacy inactive if swallowed	Imprest S8 S4R	
Prescriber signature V. Good	Print name V. Good	SAC/AAN Not supplied on D/C	

Suboxone® is available in 2 mg/0.5 mg and 8 mg/2 mg sublingual films.

Example D – Subutex® sublingual tablets			
Year 20... 23 ...		DATE AND MONTH →	
Doctors MUST ENTER administration times →			
Start Date ... 8 .. 5	Medicine (print generic name)/form Subutex Sublingual Tablets	☐ Tick if slow release	
Route Sublingual	Dose and Frequency and now enter times → 32 mg mane		
Indication CPOP	Pharmacy crush prior to administration inactive if swallowed	Imprest S8 S4R	
Prescriber signature V. Good	Print name V. Good	SAC/AAN Not supplied on D/C	

Subutex® is available in 400microg, 2mg and 8 mg sublingual tablets. Doses must be crushed using a commercial tablet crusher to coarse coffee grinds prior to administration. Crushing reduces the supervision time and risk of diversion.

3.0 Patient's own takeaway doses

- Takeaway doses brought into hospital by the patient **are not to be administered or otherwise used within the hospital.**
- Patient's own takeaway doses **cannot** be returned to the patient upon discharge **as the patient may no longer be eligible for takeaway doses after discharge** and should be reviewed by their community pharmacy and / or CPOP prescriber.
- Takeaway doses may not be provided if the patient goes on day/weekend leave. Consideration should be given to how to manage doses during this time.
- Any patient's own takeaway doses remaining after discharge must be destroyed as per local S8/S4R processes.

4.0 Ordering, Supply and Documentation

4.1 Ordering and supply of Methadone oral liquid

- Methadone oral liquid is supplied in 50 mg/10 mL pods.
- Doses are ordered:
 - At SCGH by sending a valid S8 pink requisition slip for dangerous drugs (Narcotics) to Inpatient Pharmacy between 08:30 and 15:00 on weekdays and 08:30 and 14:00 on weekends/public holidays.
 - At OPH by sending a valid S8 pink requisition slip for dangerous drugs (Narcotics) to Pharmacy 08:30 and 15:30 on weekdays, and 08:00 and 10:30 on weekends/public holidays.
- Doses must be ordered with clinical pharmacy involvement. If your area does not have a clinical pharmacy service, contact the Inpatient Pharmacy (SCGH ext 71118, OPH ext 78148).
- Order as many pods as required for up to seven doses.
 - Any liquid remaining after drawing up a dose cannot be retained. E.g. 3 doses at 25mL per dose will require 9 pods (three per dose).
 - Pre-empting weekend/public holiday requirements will ensure smooth continuation of dosing.
- See Picture 1 for an example methadone pod S8 requisition.

Picture 1: Methadone oral liquid pods label and S8 Requisition slip order

CONTROLLED DRUG
POSSESSION WITHOUT AUTHORITY ILLEGAL
KEEP OUT OF REACH OF CHILDREN

METHADONE
ORAL LIQUID
50 mg in 10mL
Use an oral (enteral) syringe to measure the required dose.

SIR CHARLES GAIRDNER HOSPITAL
Requisition for **DANGEROUS DRUGS (NARCOTICS)**
Ward or Department: **Ward GXX** **36**

Name of Preparation	Strength	Quantity or Number of Doses
Methadone oral liquid pods	50mg/10mL	9 pods

*Ordered by: **A. NURSE** (Nurse's Name in block letters) Date: **1/8/2023**
 *Supplied by: **A Nurse** (Pharmacist's Name in block letters) (Pharmacy Signature) Date:
 *Received by: (Registered Nurse's Name in block letters) (Registered Nurse's Signature) Date:
 *Please note: each of the three transactions above must be endorsed by a different staff member

4.2 Ordering and supply of Suboxone and Subutex and Buprenorphine depots

- After receiving an S8 pink requisition slip for dangerous drugs (Narcotics), supply is issued by Inpatient Pharmacy.
- Subutex® and Suboxone® films are supplied as non-imprest ward stock.
- Single buprenorphine depots will be supplied under patient name.
- Where possible, doses must be ordered with clinical pharmacy involvement. If your area does not have a clinical pharmacy service, contact the Inpatient Pharmacy (SCGH ext 71118, OPH ext 78585).
- Supply is only available if a valid and complete requisition is received by Inpatient Pharmacy between 08:30 and 15:00 on weekdays and 08:30 and 14:00 on weekends/public holidays (08:00 and 10:30 on weekends and public holidays at OPH).

4.3 Documentation of OST on wards

Follow standard procedures for S8 transaction and storage requirements per site specific guidance. Methadone pods, Suboxone® and Subutex® are recorded in the relevant S8 register.

4.4 Returning unused OST to pharmacy

- Unused doses OST sublingual tablets or films must be returned to pharmacy if the patient has had a dose change; or has been discharged.
- Unused methadone pods must be returned to pharmacy if the patient has been discharged. A request to return the doses must be written on a pink S8 requisition slip and must clearly include:
 1. Ward/area requesting return;
 2. Annotate form with "Return to pharmacy– not needed";
 3. Medicine name, form and strength and number of pods / tablets / films being returned.
 4. Reason for return; and
 5. Authorised person's printed name and signature.
- See Picture 2.
- Incomplete requisitions will be returned for completion.
- Once completed:
 - Notify your clinical pharmacist so the doses can be returned to Pharmacy; or
 - Send the requisition to Inpatient Pharmacy (Chute 84 at SCGH, or contact your clinical pharmacist directly at OPH). Two pharmacists will collect the medication on your nominated S8/S4R destruction week.

SIR CHARLES GAIRDNER HOSPITAL		ORIGINAL - PINK DUPLICATE - WHITE
Requisition for DANGEROUS DRUGS (NARCOTICS)		
Ward or Department.....	Ward GXX	36
Name of Preparation	Strength	Quantity or Number of Doses
Methadone oral liquid pods	50mg/10mL	3 pods
RETURN TO PHARMACY - NOT NEEDED		
*Ordered by A. NURSE (Nurse's Name in block letters)	 A Nurse (Nurse's Signature)	Date 3/8/2023
*Supplied by (Pharmacist's Name in block letters)	 (Pharmacist's Signature)	Date
*Received by (Registered Nurse's Name in block letters)	 (Registered Nurse's Signature)	Date

*Please note: each of the three transactions above must be endorsed by a different staff member

- Suboxone® films must be placed under the tongue (or inside the cheek – buccal if appropriate).
- The registered nurse or medication competent enrolled nurse must watch the patient place the prescribed dose underneath the tongue and supervise to ensure the dose cannot be retained in the mouth for later injection or diversion.
 - Suboxone® - for at least 1 minute
 - Subutex® - for at least 3 minutes
- Tablets and films are inactive if swallowed.
- Never leave Subutex® or Suboxone® with a patient for self-administration.

5.3 Buprenorphine Depots

WARNING: Risk of serious harm or death with intravenous administration

Serious harm or death could result if depot buprenorphine injections are administered intravenously. Sublocade® and Buvidal® form a gel or solid depot upon contact with bodily fluids and may cause occlusion, local tissue damage and thrombo-embolic events, including life threatening pulmonary emboli, if administered intravenously.

- Administration should only be necessary for patients with a prolonged admission and when discharge is unlikely to occur until after the window for their next dose. The decision to administer a dose must be made after discussion with the patient's usual CPOP prescriber or the CPOP Advice and Support service.
- Buprenorphine depots must be administered by a registered nurse.
- Staff administering buprenorphine depots are required to have watched the following videos: [Buvidal](#) and [Sublocade](#).
- Different brands are not interchangeable. Ensure the same depot product brand is used as previously administered.
- Buvidal® injection: give slowly into subcutaneous tissue of the upper arm (if sufficient tissue), abdomen, thigh or buttock; rotate injection sites.
- Sublocade® injection: remove from refrigerator at least 15 minutes before administration. Give slowly into subcutaneous tissue of the abdomen. Rotate injection sites.

6.0 Patient Monitoring

- All opioid medicines including those for OST are considered high risk medicines and require clinical monitoring.
- Even though methadone and buprenorphine are being administered for opioid pharmacotherapy rather than pain management, the patient is still at risk of developing adverse effects including respiratory depression and reduced consciousness.

7.0 Discharge from hospital

- Continuity of treatment must be confirmed and documented in the medical notes prior to discharge.
- The hospital discharge team must liaise with the authorised CPOP prescriber to ensure continuity of supply post discharge. This could include situations such as dose changes or prescription expiry during the inpatient stay.
- SCGOPHCG doctors **must not** write CPOP medication scripts for community pharmacies. Responsibility for CPOP community pharmacy prescriptions rests with the community CPOP prescriber. If unavailable, then prescriptions will be provided via CPOP Advice and Support.

- Under **no circumstances** are discharge prescriptions or 'takeaway' doses for methadone, Subutex® or Suboxone® to be provided to patients by SCGOPHCG. This includes any doses the patient presented with on admission.
- Discharge prescriptions for Schedule 8 medicines cannot legally be issued to a registered drug dependent person without prior authorisation from the Department of Health. **Contact APS for advice on managing CPOP patients with complex analgesic requirements.**
- Notify the community CPOP prescriber and/or CPP as soon as practical to advise them of any of the following patient situations during the discharge planning process:
 - The patient is unable to travel to their normal community pharmacy or Next Step due to incapacitation
 - The patient's OST script has expired
 - The patient's OST dose has been adjusted during admission

7.1 Day of discharge:

- Administer CPOP treatment on the morning of discharge.
- For patients that have had their methadone oral liquid dose split during admission under the direction of an Acute Pain Specialist, refer to the APS management plan for instructions on transitioning to the full dose or contact APS for advice, prior to the day of discharge.
- A clinical pharmacist, Alcohol and Drug CNC or ward shift coordinator must notify the community pharmacy or CPP (Next Step) of the dose amount, date and time of last dose administered in hospital, or if any doses have not been returned to the patient.
- Document notification details in medical notes to complete the clinical handover process.

7.2 Unexpected or premature discharge from hospital

- Unexpected discharge from hospital, e.g. Patient Initiated Discharge (PID), discharge against medical advice (DAMA), or discharge from hospital after hours or on a weekend/public holiday presents a challenge to continuation of OST. Under these circumstances, continuity of OST dosing is not guaranteed and in some cases will not be available.
- Where discharge under such circumstances cannot be avoided, staff are to advise the patient accordingly, and to recommend the patient contact CPP on the next workday.
- To determine if temporary dosing may be arranged, a staff member should contact CPOP Advice and Support during office hours or the Next Step CAS On-Call doctor after hours (both services may be reached on 9442 5042).

References

1. [WA Community Program for Opioid Pharmacotherapy \(CPOP\) Clinical Policies and Procedures for the Use of Methadone and Buprenorphine in the Treatment of Opioid Dependence. Community Pharmacotherapy Program. Drug and Alcohol Office. WA Mental Health Commission. 3rd ed. 2014 Clinical guidelines for use of depot buprenorphine \(Buvidal® and Sublocade®\) in the treatment of opioid dependence: For Western Australian CPOP prescribers and pharmacists. Community Pharmacotherapy Program. Mental Health Commission. 2022.](#)
2. [WA Department of Health. Risk based requirements for medicine handling](#)
3. [WA Department of Health. Guideline on continuation of opioid substitution treatment in hospitals](#)

4. [WA Department of Health. Guideline on oral liquid schedule-4 restricted and schedule 8 medicines](#)

Related Documents

Legislation:

- [Medicines and Poisons Act 2014](#)
- [Medicines and Poisons Regulations 2016](#)
- [Schedule 8 Medicines Prescribing Code](#)

Policy, Procedure and Guidelines:

- [MP 139/20 Medicines Handling Policy](#)
- [MP 77/18 Statewide Medicines Formulary Policy](#)
- [SCGOPHCG Medicines Management Policy](#) and Procedure

Authorisation & Version Control

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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